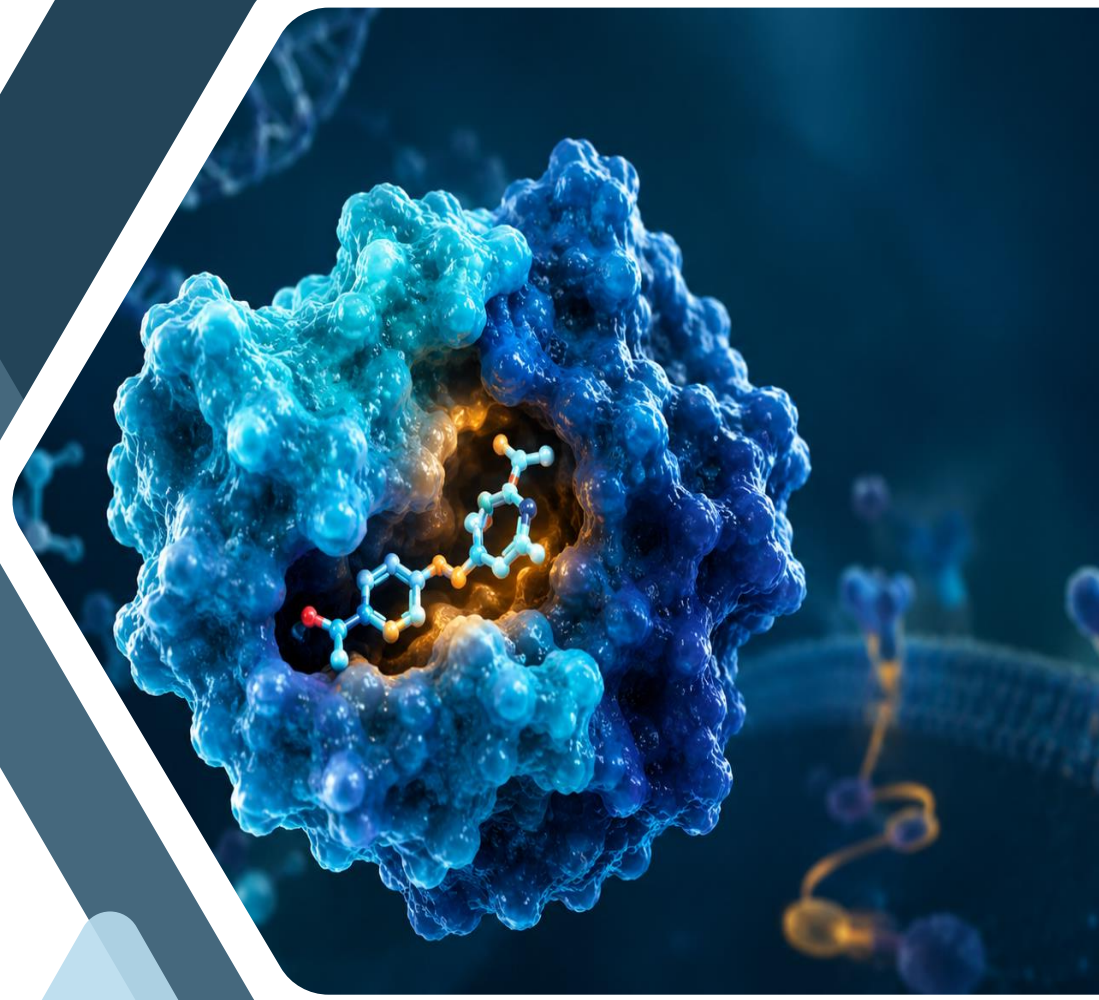
OCTAVUS CONSULTING
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REWRITING THE RULES OF RAS INHIBITOR

From an historically elusive target to a new frontier in precision oncology

Insights Corner



Month: October 2026
Presenter: Team Octavus



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- RAS Targeting Fundamentals & Therapeutic Evolution 07
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- Future Outlook & Strategic Opportunities 25

INSIGHTS CORNER

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

- Highlights from AACR-25 (**May, 25**)
- US Tariff impact on Pharma industries (**Jun, 25**)
- EULAR-25: Key insights of Psoriatic Arthritis (**Jul, 25**)
- Antibody Drug Conjugate: The next big investment in Oncology (**Aug, 25**)
- Animal Health (**Oct, 25**)
- Obesity deep dive (**Nov, 25**)
- GLP-1 and GIP (**Dec, 25**)
- Hematological Highlights from ASH 2025 (**Jan, 26**)
- China Biotech's Global Positioning from JPM-26 (**Feb,26**)
- BIOSIMILARS RISING: The Catalyst For A New Age In Biologics Therapies (**Mar,26**)
- Cell Therapy Within CGT (**Apr, 26**)
- Gene Therapy Within CGT (**May, 26**)
- Regulatory Designations US FDA & EMA (**June, 26**)
- RadioTherapy Highlights (**July, 26**)
- AI in Drug Development (**Aug, 26**)
- Companion Diagnostic (**Sep, 26**)

Additional: 1 Beautiful Bill

PRIOR ISSUES

INSIGHTS CORNER

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Executive Summary

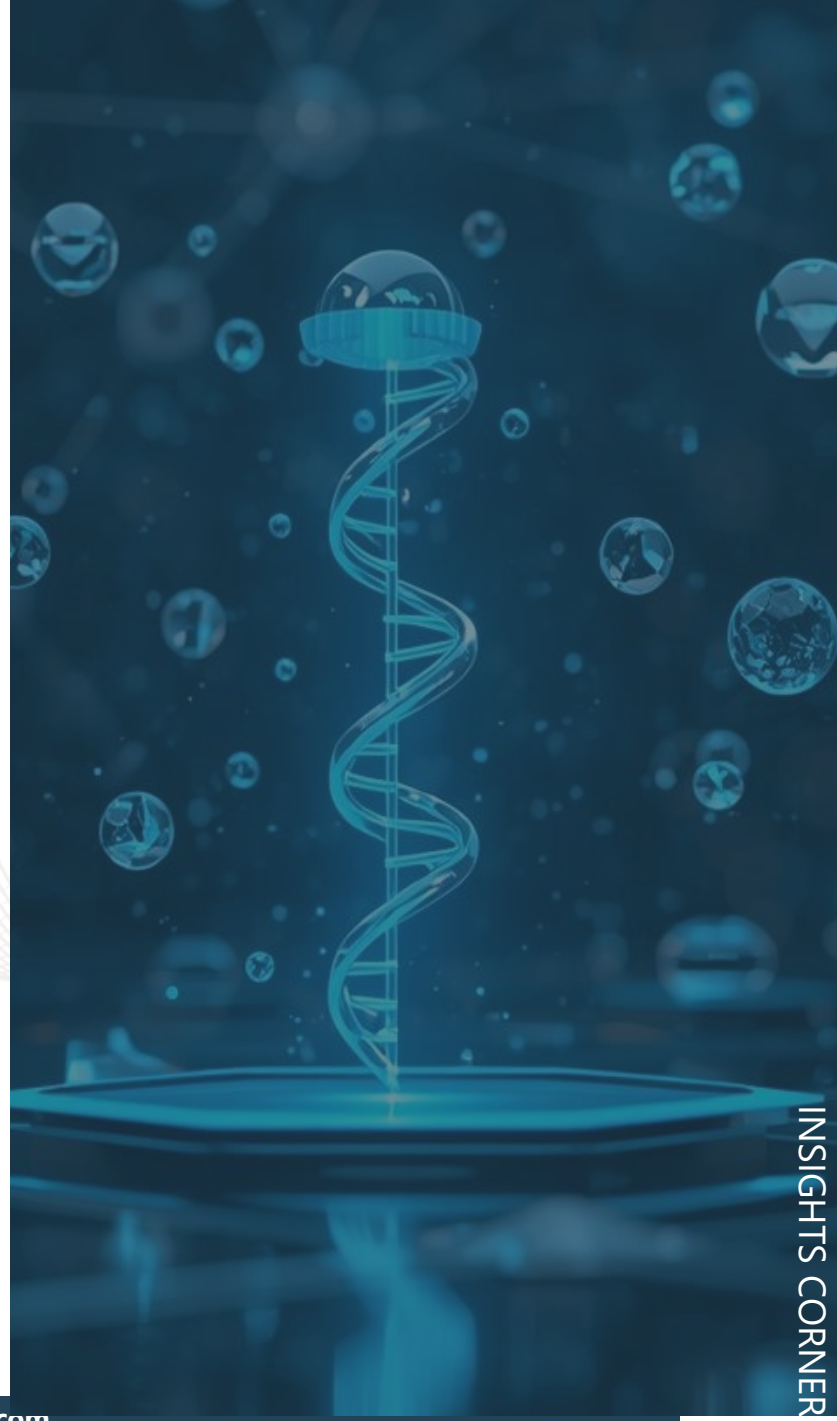
RAS has moved from "undruggable" to a validated target: RAS alterations occur in ~30% of human cancers. After KRAS G12C inhibitors (sotorasib in 2021, adagrasib in 2022), the field has moved to broader RAS(ON) approaches; Daraxonrasib (RASONQUE) received FDA approval in Aug 2026 for previously treated metastatic pancreatic adenocarcinoma; New IV modalities such as the liposomal HRS-4642 and the protein degrader ARV-806 are emerging, though their clinical differentiation is not yet proven

Clinical evidence is strong and development is accelerating: In P3 RASolute 302, daraxonrasib roughly doubled survival versus chemotherapy: mOS 13.2 vs 6.7 mos and mPFS 7.2 vs 3.6 mos, with ORR of 31.6% vs 11.2% in the overall population; Zoldonrasib + daraxonrasib showed ORRs of 50% (2L) and 47% (3L+) in KRAS G12D mPDAC. There are 13 ongoing trials in 2026, up from 1 in 2020; Early-stage studies dominate (52 of 79 trials are P1 or 1/2), and 12 trials were terminated

The competitive race is shifting to earlier lines, G12D and combinations: Daraxonrasib now sets the benchmark. Divarasib beat sotorasib/adagrasib on PFS and OS in the P3 Krascendo 1 study; In contrast, the FDA withdrew the adagrasib + cetuximab accelerated approval in CRC after KRYSTAL-10 missed both primary endpoints; The G12D space is crowded with IV inhibitors, degraders and RAS(ON/OFF) drugs, and new entrants must now compete against an approved RAS(ON) inhibitor

Revolution Medicines is the central value story: The FDA approval is the company's first proprietary product-revenue stream, following zero reported revenue in FY2024 and FY2025; It has $3.9B in cash and investments as of Jun 30, 2026; Daraxonrasib peak revenue is forecast at $15.1B (2034), with a 2027 forecast of $2.4B and a 30-day supply priced at $39,800; Analysts are bullish: Morgan Stanley is Overweight with a $255 target, and JPMorgan raised its target from $227 to $246; The deck's market-sizing illustration implies a roughly $30B opportunity (not a sales forecast), against an EV of nearly $15B

Future leadership will depend on durability, safety and earlier-line use: The outlook centers on four themes: expansion beyond G12C (G12D, G12V, pan-RAS), rational combinations, earlier-line and new-indication use, and overcoming acquired resistance; Key upcoming catalysts include RASolve 301 initial results (NSCLC, 2H 27–1H 28 window), the Divarasib Krascendo 1 full results at ESMO, and P3 starts for several G12D and pan-RAS assets; Success will hinge on proven clinical benefit and regulatory approval, not early response rates alone



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FOUNDATION
RAS Targeting
Fundamentals

RAS Inhibitors: From Oncogenic Driver to Targeted Therapy

Once considered "undruggable," RAS has emerged as a major frontier in precision oncology. Advances in structure-guided drug design have enabled direct targeting of mutant RAS, from clinically validated KRAS G12C inhibitors to emerging strategies targeting additional RAS variants and broader RAS signaling.

1 RAS BIOLOGY

RAS family: KRAS, NRAS and HRAS are small GTPases that act as molecular switches regulating cell proliferation, survival and differentiation.

Normal signaling: RAS cycles between GDP-bound inactive (OFF) and GTP-bound active (ON) states following upstream growth-factor activation.

Oncogenic activation: Mutations, predominantly at G12, G13 and Q61, can impair GTP hydrolysis and maintain RAS in an active state.

Constitutively active RAS stimulates downstream RAF–MEK–ERK (MAPK) and PI3K-AKT signaling, promoting tumor growth and survival.

2 WHY RAS IS AN IMPORTANT ONCOLOGY TARGET

RAS alterations occur in ~30% of human cancers, making the RAS pathway one of the most frequently altered oncogenic signaling systems.

KRAS is the predominant mutated RAS isoform and is particularly prevalent in pancreatic, colorectal and non-small-cell lung cancers (NSCLC).

Direct RAS inhibition was historically challenging because of the protein's high affinity for GTP/GDP and lack of obvious drug-binding pockets, leading to its longstanding reputation as an "undruggable" target.

3 THERAPEUTIC RATIONALE

RAS mutation → persistent RAS signaling → MAPK / PI3K pathway activation → tumor proliferation & survival

↓

Direct RAS inhibition

↓

Mutant RAS blockade → downstream signaling suppression → reduced tumor-cell growth

↓

Advances in structural biology and medicinal chemistry identified exploitable pockets in mutant RAS, enabling mutation-selective direct inhibitors.

4 THERAPEUTIC EVOLUTION

"Undruggable" RAS
High affinity for GTP/GDP and lack of druggable pockets

Direct KRAS Targeting
Focus on inhibiting mutant RAS activity

KRAS G12C Inhibitors
First successful mutation-selective direct inhibitors

Emerging RAS-Targeting Strategies
G12D, G12V, G12R, G13D and pan-RAS approaches

Clinical Impact
Expanding treatment options & improving outcomes in RAS-driven cancers

The first major clinical validation came with KRAS G12C inhibitors, beginning with **sotorasib (2021)** and followed by **adagrasib (2022)** in KRAS G12C-mutated NSCLC.

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

INSIGHTS CORNER

RAS Inhibitors History: Decades of Discovery Reshaping the RAS Targeting Landscape

RAS inhibitor history reflects a shift from indirect pathway blockade to direct, mutation-selective targeting; approved KRAS G12C inhibitors established clinical proof of concept, while broader RAS-targeting approaches are expanding the landscape, with efficacy and durability remaining key challenges.

PERIOD	MILESTONE	WHY IT'S MATTER
Late 1960s-1983	RAS first studied through rodent tumor viruses; human HRAS, KRAS and NRAS oncogenes identified.	Established RAS as a cancer-driving gene family.
1980s-1990s	RAS GTPase biology and signaling partners including SOS, GAPs and RAF were characterized; RAS membrane localization and farnesylation became therapeutic targets.	Explained how RAS switches on growth signaling and why it was difficult to inhibit directly.
1990s-2000s	Farnesyltransferase inhibitors and downstream pathway inhibitors were explored clinically.	Early approaches exposed limitations of indirectly targeting RAS, including alternative signaling and pathway feedback.
2013	A druggable pocket on KRAS G12C was identified, enabling covalent inhibitor development.	Provided a practical route to directly inhibit a specific mutant RAS protein.
May 2021	Sotorasib received the first FDA approval of a KRAS-targeted therapy, for previously treated KRAS G12C-mutated NSCLC.	First clinical proof that KRAS could be directly targeted with a medicine.
Dec 2022	Adagrasib received accelerated FDA approval for previously treated KRAS G12C-mutated NSCLC.	Expanded the clinical options for KRAS G12C-driven disease.
Jun 2024	Adagrasib + cetuximab received accelerated FDA approval for previously treated KRAS G12C-mutated colorectal cancer.	Demonstrated the importance of combination therapy in colorectal cancer.
Jan 2025	Sotorasib + panitumumab received FDA approval for previously treated KRAS G12C-mutated metastatic colorectal cancer.	Established another approved KRAS G12C combination approach in colorectal cancer.
2020s-2026	RAS(ON) inhibitors and broader RAS-targeting strategies advanced beyond G12C-specific inhibition.	The field began targeting multiple RAS mutants and expanding into cancers such as pancreatic cancer.
Aug 2026	Daraxonrasib (RASONQUE) received FDA approval for metastatic pancreatic adenocarcinoma after prior therapy or in patient's ineligible for multiagent systemic treatment.	A major milestone for broader RAS-family targeting beyond KRAS G12C.

INSIGHTS CORNER

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RAS Inhibitors: Beyond Oral Small Molecules to Emerging IV Modalities

Oral KRAS inhibitors established clinical proof of concept; emerging IV formulations and protein degraders are exploring alternative ways to deliver mutation-selective RAS targeting, with clinical benefit and differentiation still to be demonstrated.

1 MARKET ENTRY: ORAL WAS THE STARTING POINT

MILESTONE	MOLECULE	TARGET	FORMULATION / ROUTE	STATUS
2021	Sotorasib (LUMAKRAS)	KRAS G12C	Tablet/Oral	FDA Approved
2022	Adagrasib (KRAZATI)	KRAS G12C	Tablet/Oral	FDA Approved
2026	Daraxonrasib (RASONQUE)	RAS(ON) multi-selective	Tablet/Oral	FDA Approved

Sotorasib became the first FDA-approved direct RAS-targeted therapy on May 28, 2021, as a once-daily oral tablet. Adagrasib followed in 2022, also as an oral tablet, so the first commercial RAS inhibitors were all oral small molecules.

2 WHY THE FORMULATION LANDSCAPE MAY CHANGE

- GI tolerability / absorption challenges
- Potentially suboptimal tumor exposure
- Resistance remains major limitation of 1st-generation KRAS G12C inhibitors
- New RAS modalities may not have the same oral bioavailability characteristics

A 2026 Cancer Cell review notes that the first-generation KRAS G12C inhibitors are limited by rapid emergence of resistance, while newer mechanisms and broader RAS coverage are being pursued.

3 PROOF THAT IV RAS TARGETING IS ALREADY EMERGING

HRS-4642 - KRAS G12D INHIBITOR IV LIPOSOMAL FORMULATION

HENGRUI



- **Modality:** selective KRAS G12D small-molecule inhibitor
- **Formulation:** liposomal nanoparticle
- **Route:** IV infusion
- **Clinical stage:** Phase 1b/2
- **Population:** KRAS G12D-mutant pancreatic cancer

- ✓ **Reported in 30 previously untreated patients:** Confirmed ORR 63.3% (95% CI 43.9–80.1)
- ✓ **RP2D:** 500 mg Day 1 + 1,200 mg Day 8, every 3 weeks
- ✓ **Phase 1b:** Reported no dose-limiting toxicities

Nature Medicine describes HRS-4642 as a liposomal nanoparticle given intravenously to improve tumor accumulation & prolong target inhibition. Earlier P1 data showed weekly, every-2-week and every-3-week IV schedules, supporting intermittent dosing.

ARV-806 - KRAS G12D PROTEIN DEGRADER

ARVINAS



- **Target:** KRAS G12D
- **Modality:** Targeted protein degradation
- **Route:** IV infusion
- **Development:** Phase 1/2 first in human study
- **Clinical result:** No clinical efficacy result posted in the trial record

NCT07023731: ARV-806 is being administered through a vein (IV infusion) in a P1/2 study in KRAS G12D-mutant advanced solid tumors.

4 THE STRATEGIC TAKEAWAY

Is there scope for IV RAS inhibitors?

YES,

but primarily for next-generation modalities

OPPORTUNITY	STRATEGIC RATIONALE
Improve tumor exposure	IV/liposomal delivery may increase delivery to tumor tissue
Overcome oral bioavailability limitations	Bypasses GI absorption
Enable new modalities	Protein degraders and other non-conventional RAS approaches may favor parenteral delivery
Controlled PK	Direct systemic delivery can support defined exposure and intermittent dosing
Combination therapy	IV RAS inhibitor can potentially be integrated into existing infusion-based oncology regimens
Potential differentiation	Formulation itself could become a differentiating attribute if linked to better exposure/efficacy

INSIGHTS CORNER


EVIDENCE
Clinical Development & Evidence

RAS Inhibitors: Comprehensive Analysis of Clinical Trial on RAS Mutation from 2020 to till now

INSIGHTS

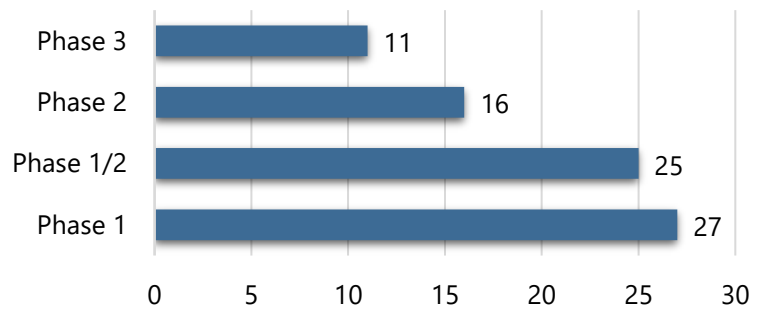
Development momentum is accelerating
13 ongoing trials in 2026
The highest annual count in the dataset, up from 1 in 2020, suggesting sustained clinical development momentum. Confirm whether 2026 represents a full year.

Early-stage development dominates
52 trials
Phase I and Phase I/II account for 52 of the 79 trials represented in the phase chart, indicating a substantial early-stage pipeline and a need to monitor progression toward later-stage evidence.

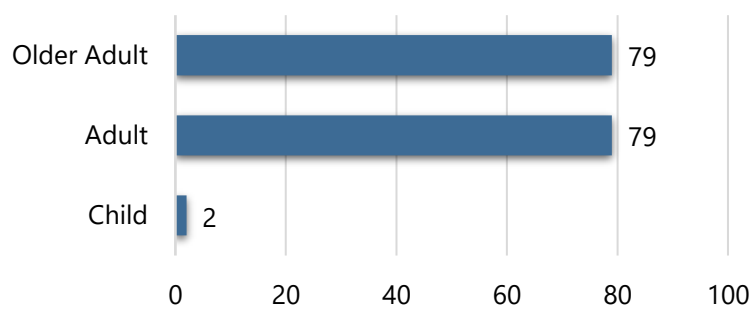
Clinical attrition warrants attention
12 terminated trials
Terminated studies represent 15% of the 79 trials in the status chart. Understanding termination reasons, efficacy signals, safety, and resistance mechanisms can help identify development risks.

Broad Adult Eligibility
79 trials Each
Adult and older-adult eligibility categories each appear in 79 entries, compared with just 2 in the child category. Because age groups may overlap within the same trial, these figures reflect eligibility coverage rather than distinct trials, highlighting the predominantly adult-focused scope of RAS inhibitor research.

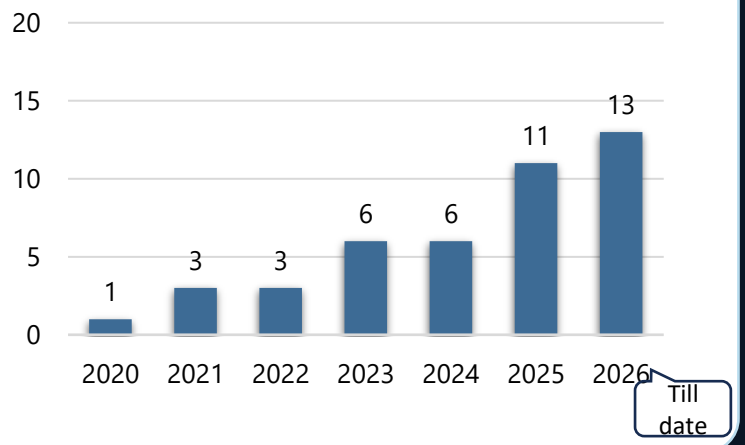
PHASE WISE



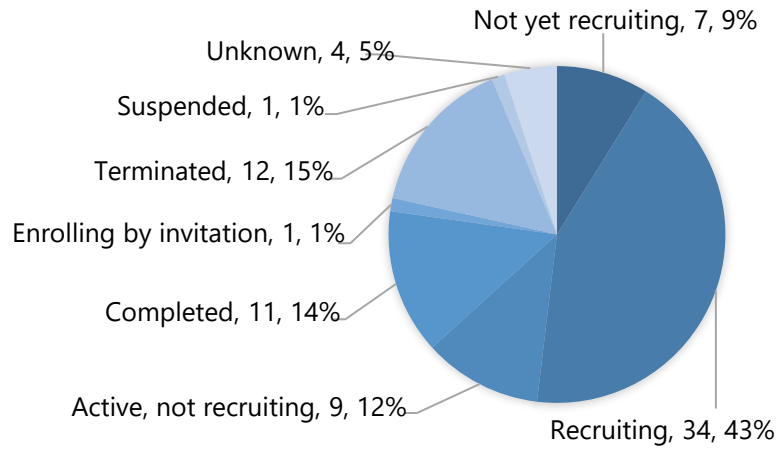
AGE WISE



YEAR WISE ONGOING TRIALS (Active Not Recruiting & Recruiting)



STATUS WISE



RAS inhibitor research is accelerating, but the real breakthroughs are yet to come. The next wave of late-phase results will decide which therapies redefine cancer treatment.

INSIGHTS CORNER

RAS Inhibitors: Expanding RAS(ON) Inhibitor Across RAS mutated Solid Tumors with Double mPFS & mOS

Mechanism:
RAS(ON) inhibitor

Focus:
RAS mutated Solid Tumors

Pipeline Extension:
1L; 2L/3L PDAC; Adjuvant PDAC

RASONQUE™ (daraxonrasib) tablets 100 mg 150 mg

RAS ASSET PROFILE

PARAMETER	RASONQUE
Modality	Small molecule
Mechanism	RAS(ON) multi-specific inhibitor
Dosage	300 mg
Biomarker	RAS mutation
Route	Oral
Setting	2L Metastatic
Combination	NA
Status	FDA approved
Pivotal study	RASolute 302
Current program	Metastatic (1L; 2L/3L); Resectable (Adjuvant)

PRICE STRUCTURE & FORECAST

Peak Revenue
$15.1B (2034)

2027 Forecast
$2.4B

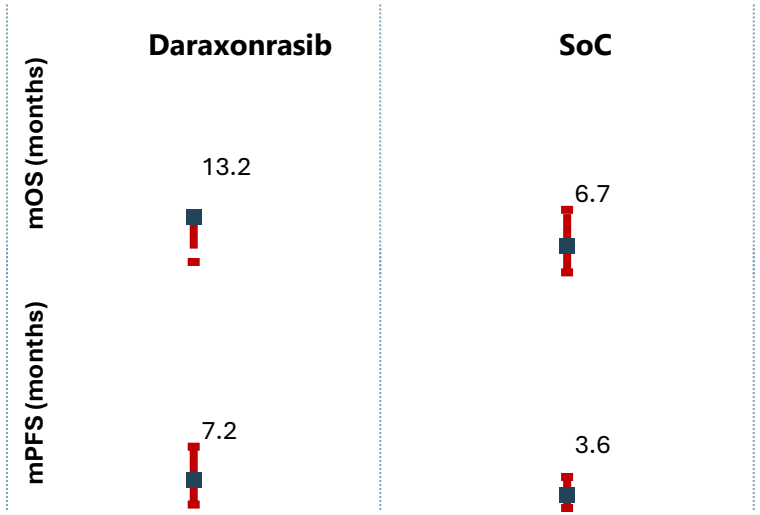
30-Day Supply
$39,800

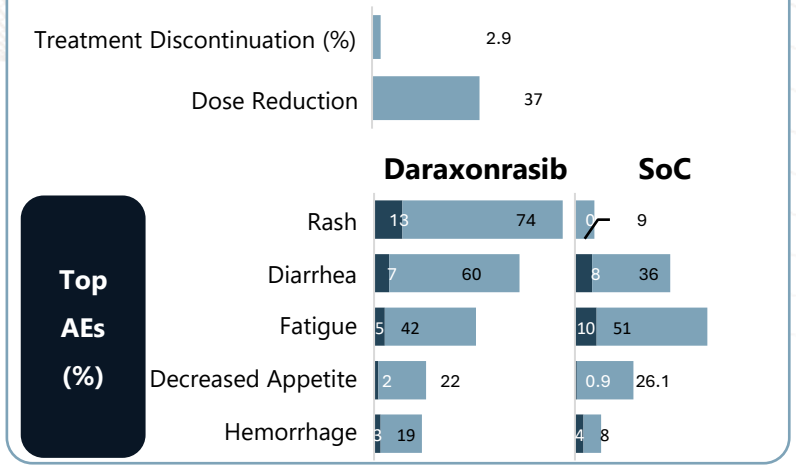
EMA Full Approval (Expected)

Mid-2027



RASOLUTE 302 EFFICACY & SAFETY





RASONQUE JOURNEY

- **Jul'26** — P3 study beings for RAS G12D PDAC
- **Jul'26** — EMA expedites under phased review process
- **Aug'26** — D&C agreement with BeOne Medicines for Asian markets
- **Aug'26** — US FDA approves Rasonque in PDAC
- **Sep'26** — Revolution publishes P1/2 RAS mut NSCLC results in *NEJM*
- **Sep'26** — RASONQUE + CT receives BTD for 1L PDAC

INSIGHTS CORNER

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Daraxonrasib: Efficacy at ASCO & ESMO 2026

Doubling Survival vs CTx; Combination Shows Promising Activity in KRAS G12D mPDAC

RASONQUE™ (daraxonrasib) tablets 100 mg 150 mg

Revolution Medicines

2026 ASCO Annual Meeting

2026 ASCO ANNUAL MEETING

Title: Daraxonrasib, a RAS(ON) multi-selective inhibitor vs chemotherapy in previously treated metastatic pancreatic adenocarcinoma (mPDAC): Primary and final analysis from the phase 3 RASolute 302 study

Data cut off Feb 10, 2026 | **mFU** 8.5 mos | **LoT** 2L | **NCT ID** NCT06625320

Efficacy		RAS G12		Overall	
		Daraxonrasib (n=228)	Chemo (n=231)	Daraxonrasib (n=248)	Chemo (n=252)
OS	Median (mos)	13.2 (10-NE)	6.6 (5.4-8.2)	13.2 (10-NE)	6.7 (5.8-8)
PFS	Median (mos)	7.3 (6.3-8.1)	3.5 (2.9-3.8)	7.2 (5.7-7.5)	3.6 (2.9-4.2)
ORR	%	33.2 (27-39.9)	11.8 (7.8-16.8)	31.6 (25.8-38)	11.2 (7.5-15.9)

2026 ESMO GI

ESMO GI 2026

Title: Safety and efficacy of zoldonrasib (RMC-9805) plus daraxonrasib (RMC-6236) in patients with 2L+ KRAS G12D metastatic pancreatic adenocarcinoma (mPDAC)

Data cut off Feb 10, 2026 | **mFU** 12.2 mos | **LoT** 2L+ | **NCT ID** NCT06040541

Population Efficacy measures	Zoldonrasib 1200 mg QD + daraxonrasib 300 mg QD	
	2L mPDAC (n=30)	3L+ PDAC (n=30)
ORR (%)	50%	47%
DCR (%)	97%	90%
6-month PFS rate (%)	71%	59%
6-month OS rate (%)	89%	82%

RESULTS AT GLANCE

Daraxonrasib monotx nearly doubled mOS (13.2 vs 6.7 mos.) and PFS (7.2 vs 3.6 mos.) vs CTx in previously treated mPDAC, with an ORR of 31.6% vs. 11.2% in the overall population. Separately, zoldonrasib + daraxonrasib achieved ORRs of 50% in 2L and 47% in 3L+ KRAS G12D mPDAC, supporting further evaluation of the combination. Promising results in both monotx and combination

INSIGHTS CORNER

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KOL DASHBOARD

Dr. Bob T. Li is a thoracic oncologist & early drug development specialist, now Senior Vice President and Global Head of Medical Affairs, Oncology at AstraZeneca, after roughly a decade at MSKCC. He trained at the University of New South Wales (MBBS) & the University of Sydney (MPH, then a PhD in Precision Cancer Medicine). His KRAS work centres on KRAS G12C and sotorasib, with 13 of his 141 publications covering trials, resistance mechanisms & long-term outcomes.



Dr. Bob T. Li,

MD, PhD, MPH, FRACP, Thoracic Oncologist & Early Drug Development Specialist

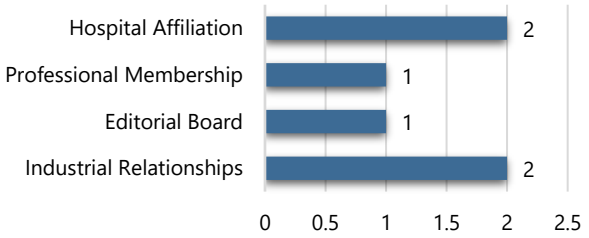
AREAS OF INTEREST

- RAS / KRAS
- Precision Oncology
- Thoracic Oncology / NSCLC
- Early Drug Development
- Liquid Biopsy & Biomarkers
- Clinical Trials

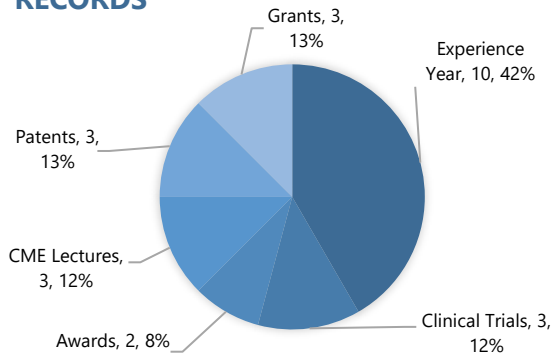
CLINICAL TRIALS



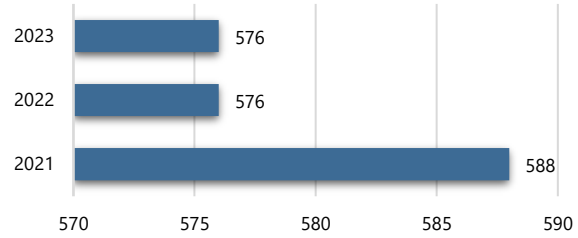
MEMBERSHIPS



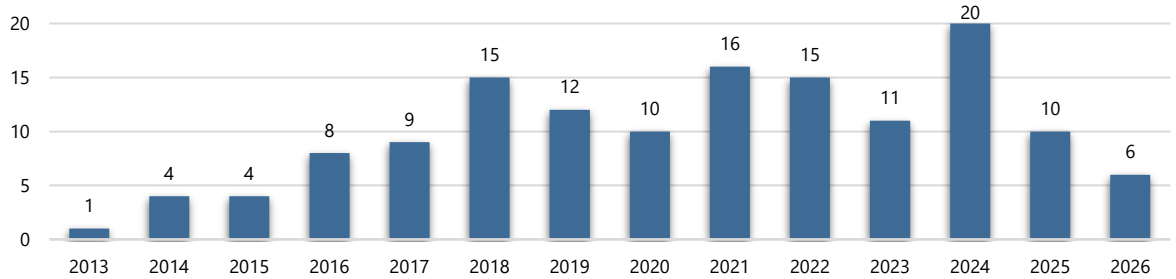
RECORDS



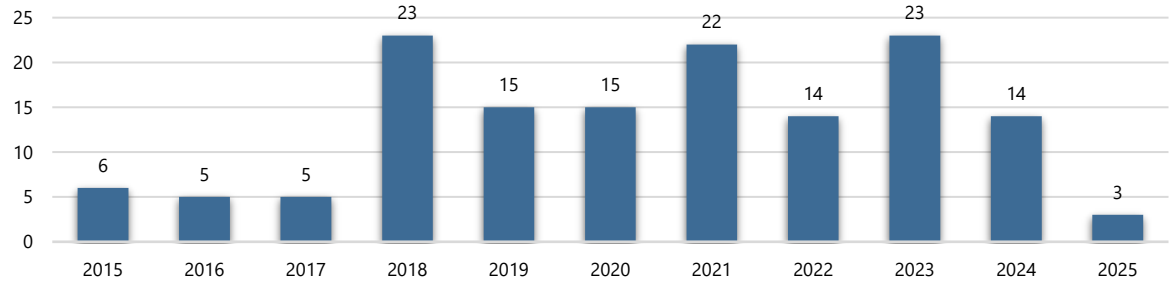
NIH GRANT FUNDING BY YEAR ($K)



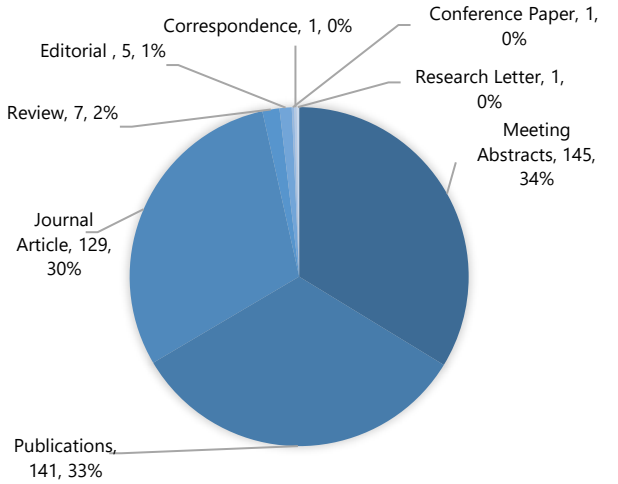
PUBLICATIONS



MEETING ABSTRACTS



RESEARCH ACTIVITIES



EXTERNAL RELATIONSHIPS

AstraZeneca

McKinsey & Company

ASIA SOCIETY

INSIGHTS CORNER

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COMPETITION
Competitive Landscape & Benchmarking

RAS Competitor Overview: Global

Indications	≤Phase 2		Phase 3/ Reg				Approved	
CRC	BLU-924 (pan-KRASi) O sanofi 1L+ (KRAS mut); MonoTx	VS-7375 (KRASG12D (ON/OFF) O VERASTEM ONCOLOGY 2L+ (KRASG12D mut; + Cetux + Panitu			Calderasib (KRASG12Ci) O MSD 1L (KRASG12C mut; +Cetuxi + mFOLFOX6.	Garsorasib (KRASG12Ci O InventisBio 盖方生物 2L+ (Mut Unknown; + FAKi		LUMAKRAS O AMGEN 2L (KRAS G12C mut; +MonoTx
NSCLC	VS-7375 (KRASG12D (ON/OFF) O VERASTEM ONCOLOGY 2L/3L & 2L-4L (KRASG12D mut); MonoTx	BLU-924 (pan-KRASi) O sanofi 1L+ (KRAS mut); MonoTx Divarasib (KRASG12Ci) O Roche 1L (KRASG12C mut; + Pembro ± CTx	Calderasib (KRASG12Ci) O MSD Adjuvant (non-sq; +Pembro SC Sosimerasib (KRAS G12Ci) O JeYou 济煜医药 1L (KRAS G12C mut non-sq); + IN10018	Calderasib (KRASG12Ci) O MSD Adjuvant (KRASG12C mut; +Pembro SC Setidegrasib (KRAS PROTAC) IV Fastellas 2L (KRAS G12D mut); MonoTx	Divarasib (KRASG12Ci) O Roche 1L (KRASG12C mut non-sq; +Pembro. Zoldonrasib (KRASG12Di) O Revolution Medicines 1L; Pembro +Pt.CTx	Divarasib (KRASG12Ci) O Roche 2L; MonoTx RASONQUE (daraxonrasib) tablets O Revolution Medicines 2L/3L (RAS mut); MonoTx	KRAZATI (adagrasib) O Bristol Myers Squibb 1L (KRAS G12C mut; MonoTx)	Jielemei (KRAS G12Ci) O JeYou 济煜医药 2L (KRAS G12D mut); Unknown LUMAKRAS O AMGEN 1L (KRAS G12C mut; +Vectibix
PDAC	BLU-924 (pan-KRASi) O sanofi 1L+ (KRAS mut); MonoTx	VS-7375 (KRASG12D (ON/OFF) O VERASTEM ONCOLOGY 2L & 1L (KRASG12D mut); ±Cetuxi	Zoldonrasib (KRASG12Di) O Revolution Medicines 1L; +Daraxonrasib	Zoldonrasib (KRASG12Di) O Revolution Medicines 1L; +mFFX/GnP	RASONQUE (daraxonrasib) tablets O Revolution Medicines Adjuvant; MonoTx JAB-23E73 (pan-KRASi) (P1b/III) O Jacobio 1L (KRAS gene Alt); +GnP	RASONQUE (daraxonrasib) tablets O Revolution Medicines 1L; ±CT Setidegrasib (KRAS PROTAC) IV Fastellas 1L (KRAS G12D mut); +mFOLFIRINOX		RASONQUE (daraxonrasib) tablets O Revolution Medicines 2L; MonoTx
Other Solid Tumors	ASP5834 (pan-KRAS PROTAC) IV Fastellas KRAS mut/ Amp ST; ± Panitumumab RLY-8161 (NRASi) O RELAY THERAPEUTICS 1L+ (NRAS mut Melanoma); MonoTx	RLY-8161 (NRASi) O RELAY THERAPEUTICS 1L+ (NRAS mut ST) ; MonoTx Setidegrasib (KRAS PROTAC) IV Fastellas KRAS G12D mut ST; ± multiple combos				RASONQUE (daraxonrasib) tablets O Revolution Medicines 2L(KRAS mut BTC); MonoTx		

Abbreviations: NSCLC: Non-small Cell Lung Cancer; Mut.: Mutation; PDAC: Pancreatic Ductal Adenocarcinoma; CRC: Colorectal Cancer; BTC: Biliary Tract Cancer

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

R: Registration; Broken border: Planned Trial

INSIGHTS CORNER

Recent Advances in RAS Inhibitors: A new benchmark, A new competitive race

Daraxonrasib's Phase 3 survival win and FDA approval in pancreatic cancer (Aug 26, 2026) make RAS(ON) inhibition the new benchmark; competition now shifts to earlier lines, G12D and combinations.

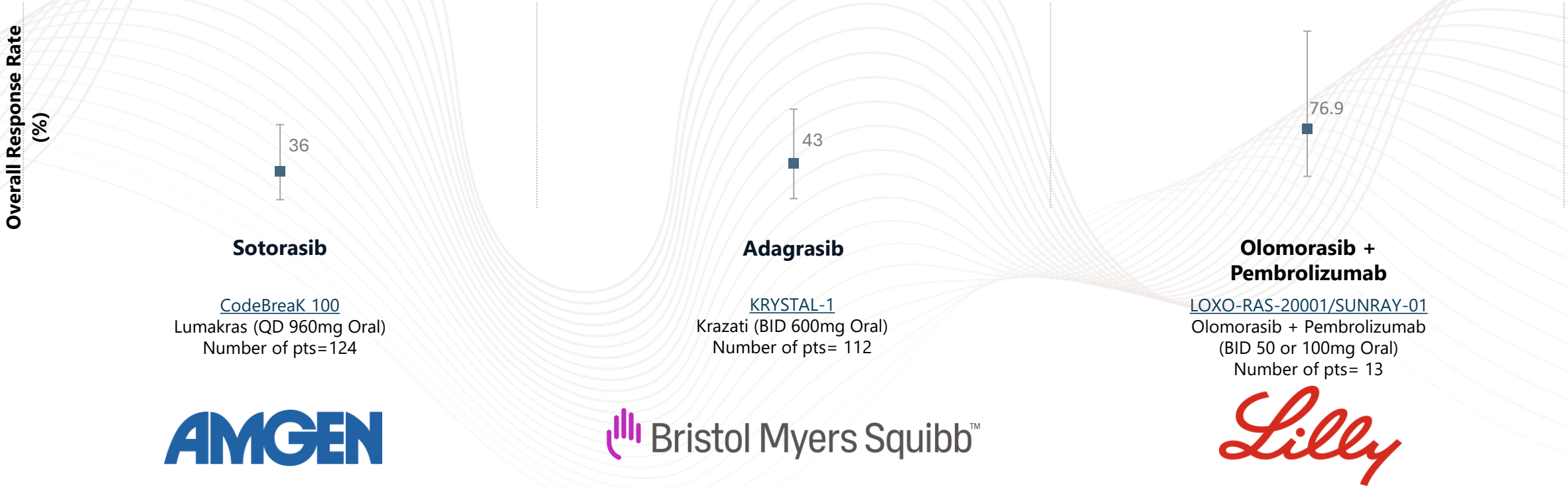
ASSET / COMPANY	ADVANCES	EVIDENCE	ANALYSIS
Daraxonrasib (RASONQUE) Revolution Medicines; RAS(ON) multi-selective; 2L PDAC **FDA Approved: Aug 26, 2026**	**Approval**	Ph 3 RASolute 302 (randomized, open-label, n=500; mFU 8.5 mo): **mOS 13.2 vs 6.7 mos, HR 0.40** vs chemo; mPFS 7.3 vs 3.5 mos (G12, HR 0.45); Gr≥3 AEs 61.8% vs 69.6%. Rash and stomatitis are key AEs	**First RAS inhibitor approved for pancreatic cancer (PDAC):** Benefits may extend beyond G12 mutations, which account for 40-50% of all PDAC patients. Sets a new benchmark for future PDAC treatments
Daraxonrasib Revolution Medicines; 1L PDAC; NSCLC **BTD; Ph 3 Ongoing**	**BTD beyond G12C**	BTDs: 1L mPDAC (+gem/nab-P, Ph1/2) and 2L KRAS non-G12C NSCLC; Ph 3 RASolute 301 study ongoing	**Expands into earlier treatment settings and NSCLC:** RASolve 301 is the next key milestone. Non-G12C mutations account for ~60% of RAS-mutant NSCLC, creating opportunities beyond G12C-specific drugs.
Divarasib Roche; KRAS G12C (OFF); 2L NSCLC **Ph 3 Topline data**	**Ph 3 results**	Krascendo 1 (randomized, open-label, n=338) vs sotorasib or adagrasib: **Beat sotorasib/adagrasib on PFS and OS**	**First direct win against another G12C drug;** Could challenge sotorasib and adagrasib in 2L treatment if the benefit holds. The size of the benefit, brain activity (CNS), and tolerability remain to be confirmed
Zoldonrasib Revolution Medicines; RAS(ON) G12D-selective; NSCLC, PDAC **BTD; Ph 3 Ongoing**	**BTD**	First BTD for a G12D-specific agent: 2L G12D NSCLC; Ph 3 studies ongoing: RASolve 308, RASolute 305, 309	**Sets a new benchmark for G12D treatment** with an oral RAS(ON) inhibitor. The zoldonrasib + daraxonrasib combination could offer a chemotherapy-free option for pancreatic cancer, but evidence is still limited and early
G12D fast-followers HRS-4642 (Hengrui); VS-7375 (Verastem/GenFleet); setidegrasib (Astellas) **Ph1/2; Early Signals**	**Ongoing early phases beyond G12C**	HRS-4642 + adebrelimab, previously treated PDAC (Ph1b/2, n=48): **ORR 41.9%**, mPFS 5.6 mos.; VS-7375 (ON/OFF) US Ph1/2a ongoing	**Highly competitive G12D landscape (~40% of PDAC cases),** with IV inhibitors, degraders and RAS(ON/OFF) drugs in development. Single-arm response rates cannot be directly compared, and new entrants must now compete against an approved RAS(ON) inhibitor
Adagrasib + cetuximab BMS (Krazati); KRAS G12C; 2L CRC **Ph 3; Accelerated approval withdrawn**	**Accelerated approval withdrawn**	KRYSTAL-10 (randomized, n=461) **missed both primaries**: mPFS 7.5 vs 8.1 mos (HR 0.89); mOS 21.6 vs 21.7 mos (HR 0.83); ORR 47% vs 16%. FDA withdrew the 2024 accelerated approval (effective 1 Sep 2026); NSCLC label unaffected	**Tumor response did not lead to improved survival** versus standard chemotherapy. G12C + EGFR combinations in colorectal cancer need further evidence

Efficacy Benchmarking: Emerging ORR advantage in previously treated NSCLC

Olomorasib + pembrolizumab shows the highest reported ORR (76.9%) in previously treated NSCLC, versus adagrasib (43%) and sotorasib (36%); however, the combination's small sample size (n=13) limits cross-trial comparability

Previously Treated NSCLC Efficacy Data

ORR



Sotorasib

CodeBreaK 100
Lumakras (QD 960mg Oral)
Number of pts=124

Adagrasib

KRYSTAL-1
Krazati (BID 600mg Oral)
Number of pts= 112

Olomorasib + Pembrolizumab

LOXO-RAS-20001/SUNRAY-01
Olomorasib + Pembrolizumab
(BID 50 or 100mg Oral)
Number of pts= 13

INSIGHTS CORNER

Safety Benchmarking: Distinct toxicity profiles across KRAS G12C inhibitors

Attributes (%)	Lumakras	Krazati	Olomorasib
	Sotorasib	Adagrasib	Olomorasib + Pembrolizumab
	CodeBreaK 100 Lumakras (QD 960mg Oral) Number of pts=124	KRYSTAL-1 Krazati (BID 600mg Oral) Number of pts= 112	LOXO-RAS-20001/SUNRAY-01 Olomorasib + Pembrolizumab (BID 50 or 100mg Oral) Number of pts= 13
Treatment discontinuation (%)	9	13	15.4
Dose reduction or interruption (%)	34	28	
Deaths (%)	3.4	11	
Top 10 AEs (%)			
Hepatic Toxicity	12, 13	10, 27	15.4
Diarrhea	5, 37	0.9, 69.1	7.7
Neutropenia	0, 0	0, 0	0
Nausea	1, 25	4.3, 64.7	
Vomiting	1.5, 15.5	0.9, 55.1	
Musculoskeletal pain	8, 27	7, 34	
Cough	1.5, 18.5	0.9, 23.1	
Dyspnea	2.9, 13.1	10, 25	
Fatigue	2, 24	7, 52	
Pneumonia	7, 5	17, 7	

Gr 1-2 Gr≥3

INSIGHTS CORNER


STRATEGY
Company &
Investment
Perspectives

Revolution Medicines: Unlocking the Future of RAS-Targeted Cancer Therapy

OVERVIEW

Founded	2014
CEO	Mark A. Goldsmith, M.D., Ph.D.
Focus	Oncology - Precision Medicines for RAS-driven Cancers
Ticker	NASDAQ: RVMD
Financial Position	$3.9B In Cash And Investments As Of Jun 30, 2026

UPCOMING MILESTONES

RASolve 301 • Daraxonrasib (P3)
Previously treated RAS-mutant NSCLC
Enrollment completes 2026; readout 2027
RASolute 303 • Daraxonrasib (P3)
1L metastatic PDAC
Est. primary completion: Jun 2028
RASolute 304 • Daraxonrasib (P3)
Adjuvant PDAC
Est. primary completion: May 2029
RMC-5127 • G12V inhibitor (P1)
RP2D expected H2 2026; initial data 2027
RM-055 • Preclinical
First-in-human trial planned Q4 2026

PARTNERSHIP

- ❑ Evaluate combinations with 4 RAS(ON) inhibitors (daraxonrasib, zoldonrasib, elironrasib, RMC-5127)
- ❑ Exclusive Asian rights (BeOne)
- ❑ Milestones + tiered royalties

Revolution Medicines × BeOne

RAS-TARGETED PIPELINE

MOLECULE	EARLY CLINICAL	PHASE 1 / 2	PHASE 3	APPROVED	STATUS / NOTES
Daraxonrasib RMC-6236 · RAS(ON) multi-selective	■	■	■	■	US-approved for specified metastatic PDAC population
Zoldonrasib RMC-9805 · RAS(ON) G12D-selective	■	■	■		P3 programs in PDAC; NSCLC program advancing
Elironrasib RMC-6291 · RAS(ON) G12C-selective	■	■			Early clinical; mono- and combo studies
RMC-5127 RAS(ON) G12V-selective	■				First-in-human trial-initiated Jan 2026
RMC-0708 RAS(ON) Q61H-selective	■				Emerging mutant-selective program
RMC-8839 RAS(ON) G13C-selective	■				Emerging mutant-selective program
RM-055 Next-generation RAS(ON) inhibitor class	■				IND-enabling / preclinical; not clinical-stage

RECENT DEVELOPMENT UPDATES

RAS(ON) Portfolio
Daraxonrasib, RMC-6291 (G12C) and RMC-9805 (G12D) show clinical activity and safety across RAS-mutant tumours.

DARAXONRASIB | P3 | RASolute 302 | Met Endpoints
Primary Endpoints: 60% lower risk of death; median OS 13.2 vs 6.7 mos with chemo, supporting global regulatory submissions.

DARAXONRASIB | P1/2 | 1L Pancreatic
Reported ORR: 47% monotherapy; 58% with gemcitabine/nab-paclitaxel, supporting P3 RASolute 303.

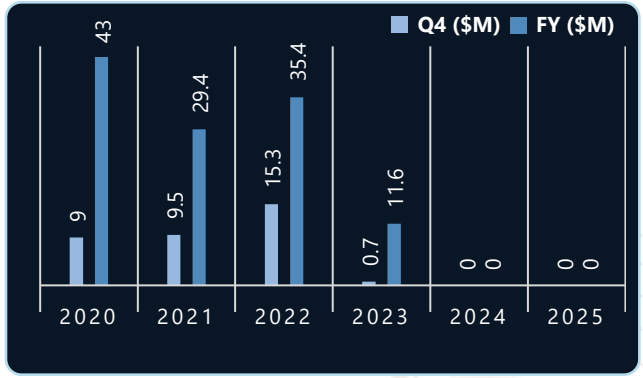
RASONQUE + GEM/NAB-PACLITAXEL
FDA Breakthrough Therapy Designation for 1L metastatic PDAC; supporting ongoing P3 RASolute 303.

ZOLDONRASIB + CHEMO | RASolute 305
Phase 3 dosing started in 1L KRAS G12D metastatic pancreatic cancer; PFS and OS are primary endpoints.

DARAXONRASIB | P1/2 NSCLC
NEJM results showed antitumour activity and durable responses; data supported Phase 3 development.

ZOLDONRASIB | P1 | NSCLC
52% confirmed ORR, 93% disease control rate and 11.1 months median PFS in previously treated KRAS G12D NSCLC.

Q4 VS. FULL-YEAR REVENUE



STRATEGIC GROWTH TIMELINE

2023: Sanofi Collaboration Ends
Collaboration income stops, leaving the company dependent on external partners

↓

FY2024 & FY2025: Zero Revenue
No revenue reported, highlighting reliance on collaboration income.

↓

Aug 2026: RASONQUE FDA Approval
Strategic turning point: the company's first proprietary product-revenue stream.

↓

Next Phase: Opportunities
Reduce reliance on collaboration income, monetize RAS-targeted innovation and build a sustainable commercial oncology business.

↓

Going Forward: Growth Drivers
Drive RASONQUE uptake, expand into new treatment settings and advance the broader RAS inhibitor pipeline.

INSIGHTS CORNER

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

RAS Inhibitors: Investor Analyst View

Revolution Medicines' nearly $15 billion EV could look modest if daraxonrasib secures first-line use and approaches its commercial potential. With 150,000+ annual pancreatic cancer cases across G8 countries, assumed 90% eligibility and $250,000 pricing imply a roughly $30 billion market opportunity—not a sales forecast. The thesis hinges on clinical performance, frontline adoption and achievable pricing, alongside rumored acquisition interest

STIFEL

Morgan Stanley

Initiated at Overweight with a $255 target. Viewed Revolution as a leading RAS-targeted oncology franchise, with approximately $12 billion in modeled peak risk-adjusted portfolio sales and $18 billion unadjusted by 2036. The reported thesis covers pancreatic, lung, and colorectal cancers, with Zoldonrasib identified as a potential next commercial opportunity.

Reportedly raised the target from $227 to $246 and maintained a bullish rating, described by the article as "Buy." The publicly accessible summary does not provide a detailed JPMorgan-specific sales model.

J.P.Morgan

What The Analyst Commentary Suggests Investors Are Evaluating

Peak Sales And Market Expansion

Can a RAS inhibitor become a multi-indication franchise across pancreatic, non-small-cell lung, and colorectal cancers, rather than a single-indication product?

Peak Sales And Market Expansion

Can multi-selective RAS(ON) inhibitors and mutation-selective inhibitors, including combination regimens, outperform or complement existing KRAS G12C approaches?

Clinical Differentiation

How do response rates, DoR, OS, tolerability, and treatment discontinuation compare with CTx and other targeted therapies?

Peak Sales And Market Expansion

How much of the projected commercial opportunity is already reflected in the company's valuation, and how sensitive is it to clinical results, safety, competition, and launch uptake?

INSIGHTS CORNER

The RAS Race: Upcoming milestones & value drivers

2026

Oct 25-26
INCB161734 (Incyte; KRAS G12D): Safety and efficacy data will be presented at ESMO 2026

Oct 25
Divarasib (Roche; KRAS G12C): Full results (PFS) of Ph 3 Krascendo 1 will be presented at ESMO

Q4
Elironrasib (Revolution Medicines; RAS(ON) G12C): **Ph 3 RASolve 307** will be initiated for 1L NSCLC

Q4
RM-055 (Revolution Medicines; RAS(ON)): First in human study will be initiated

2H
ERAS-4001 (Erasca; pan-KRAS): Expected to release **Ph 1 BOREALIS-1** preliminary monotherapy results

End of year
- **VS-7375** (Verastem; KRAS G12D): Expected to complete enrolment of **Ph 2 TARGET-D 201/202/203** studies
- Expected to meet with US FDA to review **Ph 3** trial designs in 1L mPDAC, 1L mCRC and 1L NSCLC

2027

- **Daraxonrasib** (Revolution Medicines; RAS(ON)): Expected to release initial results of **Ph 3 RASolve 301** for previously treated NSCLC
- **RMC-5127** (Revolution Medicines; RAS(ON) G12V): Expected to release initial clinical results of **Ph 1** study
- **ERAS-4001** (Erasca; pan-KRAS): Expected to initiate monotherapy expansion and combination dose escalation cohorts
- **ERAS-0015** (Erasca; pan-RAS): Expected to initiate **Ph 3** trial for 1L PDAC

1H
- **ERAS-0015** (Erasca; pan-RAS): Expected to release **Ph 1 AURORAS-1** monotherapy expansion and combination dose-escalation results
- Expected to initiate registration-enabling trial in 2L+ NSCLC

1H
VS-7375 (Verastem; KRAS G12D): First patient will enroll in **Ph 3** pivotal trial for 1L mPDAC, 1L mCRC and 1L NSCLC

2027+

2H 27-1H 28
ERAS-0015 (Erasca; pan-RAS): Expected to initiate **Ph 3** trial for NSCLC



Evidence cut-off Oct 13, 2026

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com


OUTLOOK
Future Outlook &
Strategic
Implications

Future Outlook: RAS inhibition has moved from proof-of-concept to standard of care

Oncology RAS inhibitors are evolving beyond KRAS G12C toward broader mutation coverage, rational combinations, and resistance management. Future competitive leadership will depend on durable efficacy, manageable safety, and proven clinical benefit, with successful trials potentially enabling earlier-line use and stronger market positioning.

		Evidence
1	**Expansion Beyond KRAS G12C** Development of KRAS G12D-selective, multi-selective KRAS and pan-RAS inhibitors could address a broader range of RAS-mutant tumors, especially pancreatic and colorectal cancers.	G12D agents enter Ph 3: zoldonrasib + chemo 1L PDAC (RASolute 305); Incyte INCB161734 1L (DAWN-303). G12V (RMC-5127) in Ph 1
2	**Shift Toward Rational Combination Therapies** Combining RAS inhibitors with EGFR, SHP2, SOS1, or downstream pathway inhibitors may improve response depth and durability. In colorectal cancer, EGFR co-inhibition is already an established strategy for KRAS G12C-targeted treatment.	1L NSCLC: Elironrasib combo demonstrated confirmed ORR 85%, DCR 97%, 95% progression free at 6 months (N=39)
3	**Earlier-Line Treatment & Indication Expansion** Positive clinical trial results may enable RAS inhibitors to be used earlier in treatment and across more cancer types. Success will depend on proven efficacy, safety, and regulatory approval, not just early response rates.	Daraxonrasib: Recently approved by US FDA for 2L mPDAC with OS 13.2 vs 6.7 months (chemo), HR 0.40
4	**Overcoming Acquired Resistance** Tumors can develop mutations or activate alternative pathways that reduce treatment effectiveness. Next-generation inhibitors, protein-degrading drugs, and biomarker-guided combination therapies may help overcome these resistance mechanisms.	Within-class competition: Divarasib beat sotorasib/adagrasib on PFS and OS in Ph 3 Krascendo1 study

INSIGHTS CORNER

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ABOUT COMPANY

Research & Intelligence that Proves our ROI

Competitive Intelligence & Monitoring

- Real-time CI monitoring
- Strategic early warning
- War-gaming & Simulation workshops
- Conference coverage
- Trial Optimization Strategy
- Digital Biomarker Strategy
- Competitor Promotion tracking

Portfolio & Commercial Strategy

- Opportunity Scanning
- Portfolio Strategy & Asset Prioritization
- Brand positioning
- Peak Sales Forecasting
- Digital therapeutic evaluation
- Innovation pipeline screening

Business Development & Licensing

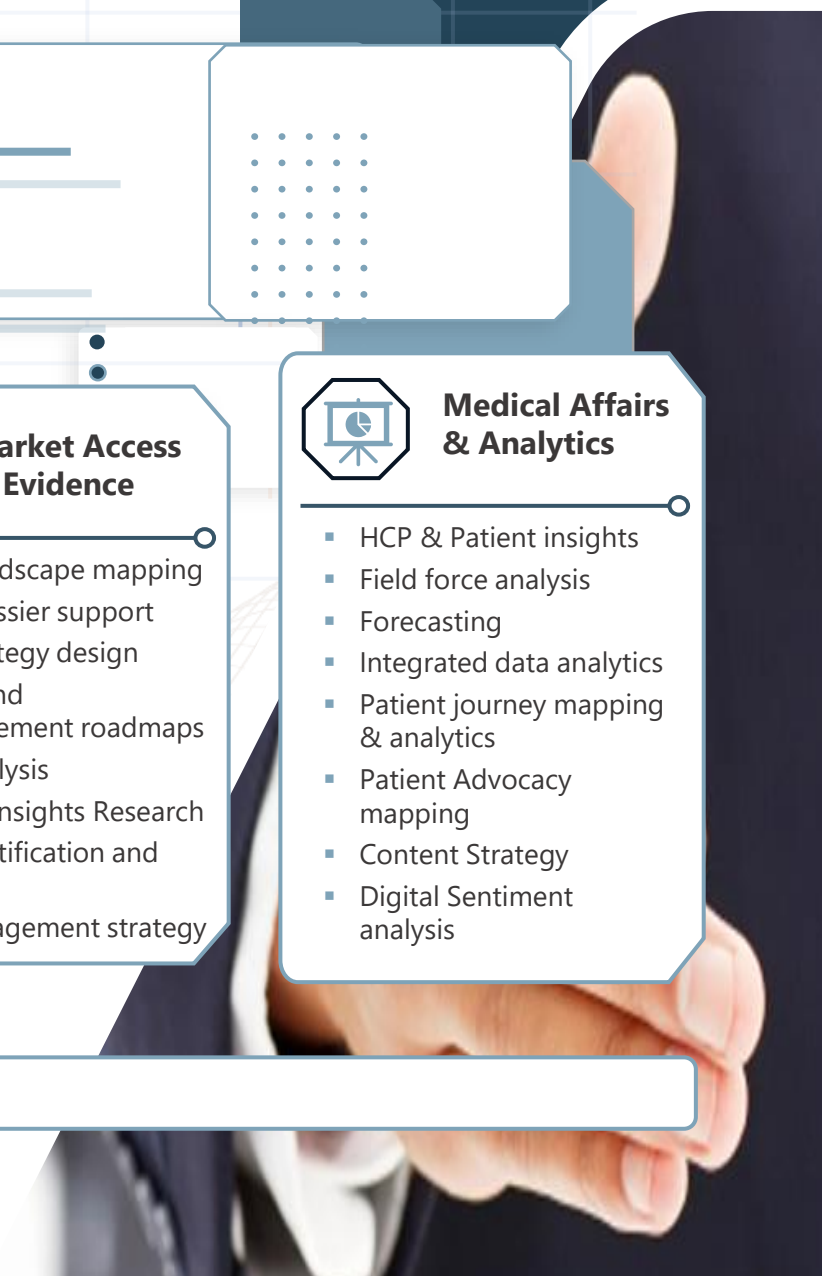
- Target identification and validation
- Retrospective deal analysis
- Synergy assessment
- M&A due diligence
- Digital health and innovation advisory

Market Access & Evidence

- Payer landscape mapping
- Value dossier support
- RWE strategy design
- Pricing and reimbursement roadmaps
- RWE Analysis
- Medical Insights Research
- KOL identification and mapping
- KOL Engagement strategy

Medical Affairs & Analytics

- HCP & Patient insights
- Field force analysis
- Forecasting
- Integrated data analytics
- Patient journey mapping & analytics
- Patient Advocacy mapping
- Content Strategy
- Digital Sentiment analysis



Capabilities are configured to fit the decision, asset stage and timeline.

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Thank You,
You Can Contact Us



Himanshu Sehgal
himanshu@octavusconsulting.com
Managing Partner



Rakesh Singh
rakesh@octavusconsulting.com
BD Manager

